Pivotum

THE DEGREE RISK INDEX · FALL 2026 EDITION · FREE SAMPLER

Is Dentistry Safe From AI? The 2026 Degree Risk Score

The free sampler — the score by track, the six factors in brief, and the honest reasons to think twice. The complete profile goes deeper.

2.7

AI EXPOSURE · GENERAL DENTISTRY

where 10 is most at risk

Re-scored every six months. We publish where we might be wrong.

HIGHLIGHTING IS COLOUR-CODED

THE FINDING EXPOSURE PROTECTION METHOD & HONESTY

Quick answer: General dentistry scores **2.7 out of 10** for AI exposure, where 10 is most at risk — among the lowest in this index. Oral surgery scores **2.3**. But dental laboratory work scores **5.5**, because the profession quietly automated its own supply chain a decade before anyone was talking about AI.

The short answer for parents: yes, and dentistry is routinely overlooked by families who say "medicine" without realizing it is a separate route. Precise physical work on an anxious person, comprehensively licensed, with practice ownership as a realistic destination. The genuine trade-offs are physical wear and running a business, not automation.

Dentistry AI risk score by role

Every career in this index is scored 1–10, where **10 is most exposed** to AI. Same six factors, same weights, applied identically to a dentist and a paralegal.

ROLE	2023	2025	FALL 2026	3-YR MOVE	BAND
Oral & maxillofacial surgery	1.9	2.1	**2.3**	+0.4	Very low
General dentistry	2.3	2.5	**2.7**	+0.4	Very low
Dental hygienist	2.6	2.8	**3.0**	+0.4	Low
Orthodontics	2.9	3.1	**3.4**	+0.5	Low
Dental technician / laboratory	4.6	5.1	**5.5**	+0.9	Moderate

2023 and 2025 figures are reconstructed using current methodology, not archived from past editions.

How that compares: the median career in this edition scores around 5.5. Surgery scores **2.1**. Bedside nursing scores **2.8**. Pharmacy scores **4.9**. Entry-level software development scores **8.1**.

Oral surgery at 2.3 is among the three lowest scores in the entire index, alongside general surgery and midwifery.

Will AI replace dentists?

Not the hands. It is already substantial in diagnosis, planning and — most of all — fabrication.

AI IS TAKING	IT CAN'T TOUCH
X-ray and scan analysis	Working in a small space, in a moving mouth
Treatment planning and simulation	Managing a frightened patient
Crown, aligner and prosthetic design	Physical procedures on live tissue
Scheduling, recalls and billing	Judgment when the tooth is worse than the image showed
Clinical note generation	Accountability for a procedure that goes wrong
Materials and inventory management	Running a practice as a business

Why does dentistry score so low? The six factors

How general dentistry rates against each. Ratings are 0–10 on each factor's own terms.

FACTOR	RATING	EFFECT ON RISK
How much of this job can AI already do?	5.0	↑ moderate exposure
How hard will it be to land that first job?	2.5	↓ strong protection
Does it have to be done in person, with your hands?	9.5	↓ strongest in the index
Does someone need a human they can trust and hold responsible?	8.5	↓ strong protection
Does the law require a licensed human?	9.5	↓ strongest available
How often does the job hit genuinely new, high-stakes situations?	7.5	↓ good protection

ONE FACTOR, WORKED THROUGH IN FULL

So you can see what the analysis actually looks like.

How much of this job can AI already do? — rated 5.0, and dentistry already ran this experiment on itself

Most professions in this index are early in their automation story. Dentistry is not, and that makes it unusually informative — because the part of dentistry that could be automated already has been, and we can see what happened.

The dental laboratory is the case study. Crowns, bridges, dentures and aligners were once made by hand by skilled technicians working from physical impressions — a genuine craft with a long apprenticeship. Over roughly fifteen years, intraoral scanning, CAD design and milling or 3D printing took most of it. That transformation was well underway before generative AI existed, and AI has accelerated the design step rather than started the process.

That is why dental technician work scores 5.5 while the dentist scores 2.7. Same industry, same building in some practices, entirely different exposure — and the divergence happened decades before anyone was writing articles about AI and jobs.

The lesson for anyone reading this index is about what actually determined the outcome. It was not skill: dental technicians were highly skilled. It was not training length. What mattered was that the work was *fabrication to a specification*, performed away from the patient, on an object rather than a person. Every one of those properties made it automatable.

The dentist's work has the opposite properties at every point. It happens on a live, moving, anxious person; the specification changes when the drill reveals what the scan could not; and a licensed human is legally accountable for the outcome.

The general lesson: look for a profession that has already partially automated, and check which part went. It tells you far more than a forecast does, because it is evidence rather than prediction. In dentistry the answer is unambiguous — the object-making went, and the person-treating did not.

Which dental careers are safest from AI?

Ranked by exposure, safest first:

1. **Oral & maxillofacial surgery — 2.3.** Surgical, high-consequence, comprehensively licensed. 2. **General dentistry — 2.7.** Manual work on a live patient with legal accountability. 3. **Dental hygienist — 3.0.** Hands-on and licensed, with more protocol-driven content and a much shorter training route. 4. **Orthodontics — 3.4.** Highly effective, but treatment planning and aligner design are among the most automated areas in dental medicine. 5. **Dental technician / laboratory — 5.5.** Largely automated already.

Is dentistry a good career in 2026?

On AI exposure it is close to settled. The genuine considerations are different.

In its favor: high autonomy, practice ownership as a normal rather than exceptional destination, strong earnings, immediate visible results within a single appointment, and predictable hours compared with most of medicine.

Against: most patients are anxious and managing that all day is a real skill nobody warns you about. The work is physically repetitive in a fixed posture, and neck, back and shoulder problems are common enough to end careers. Training is five years or more, and running a private practice means running a business — with the commercial pressures that brings.

Worth knowing about the hygienist route: shorter training, hands-on patient care, and a score of 3.0. For someone drawn to the clinical work rather than to diagnosis, surgery and business ownership, it is a genuinely different proposition rather than a lesser version of the same one.

Common questions

Will AI replace dentists? No. The work is manual, performed on a live patient whose condition changes as you treat it, and legally reserved to a licensed human.

Is dentistry a good career? On exposure, one of the best available at 2.7. The real trade-offs are physical wear over decades and the demands of running a practice.

What dental jobs are safest from AI? Oral and maxillofacial surgery at 2.3, then general dentistry at 2.7. Both are surgical or procedural work on a person.

Is dental technician work at risk? It has already substantially automated — CAD design and milling replaced most hand fabrication over the past fifteen years. It scores 5.5, by far the most exposed part of the field.

Is dentistry safer than medicine? Comparable — general dentistry 2.7 against patient-facing medicine 2.9. Dentistry has a much shorter path to independent practice and no equivalent of radiology's exposure.

Related profiles

- Medicine — 2.1 surgery to 4.9 radiology
- Nursing — 2.8 bedside, 5.4 desk-based

- **Physical therapy & allied health** — 2.5, among the lowest in this edition
- **Veterinary medicine** — 2.7 small animal practice
- **Pharmacy** — 4.9 community, 4.4 specialist clinical

What's in the full dentistry profile

This sampler tells you where dentistry stands. The full profile tells you what to do about it.

	SAMPLER	FULL PROFILE
Verdict, all role scores, 3-year trend	✓	✓
Six-factor ratings	✓	✓
Reasoning behind every factor rating	one example	✓
How durable each protection is		✓
The honest downsides — physical toll, patient anxiety, business pressure		✓
What's genuinely good about it — and who this work suits		✓
The AI-native advantage — how to prepare, and why this cohort has an opening		✓
Routes in — dentistry, hygiene, therapy, and their timelines		✓
Where it leads later — specialization, ownership, teaching		✓
Program evaluation checklist — clinical hours, patient volume, outcomes		✓
Questions to ask a program — plus red flags		✓
Sourced further reading		✓
Discussion questions for parent and student		✓
A short version written directly to the student		✓
Technical scoring appendix		✓

GET THE FULL PROFILES

Most families are weighing two or three careers seriously, and a few more they haven't ruled out. Pick the ones you need.

1 profile	**$19**	Choose →
3 profiles	**$29**	Choose →
5 profiles	**$39**	Choose →

Each full profile includes everything in the table above, plus a short version written directly to the student and the technical scoring appendix. Spring 2027 updates of whatever you buy are included.

Read a complete profile free → We publish one profile in full, openly, so you can judge the depth before buying anything. Computer science is the one to read — it's the most surprising result in the index.

Pivotum scores thirty careers against AI exposure using a fixed, published methodology, re-scored every six months. Scores measure exposure to what AI can already do — not how much any particular employer has deployed. We publish where we might be wrong.